glutamate receptor, ionotropic, AMPA 2 [*Rattus norvegicus*(Norway rat)]	Chromosome 2, NC_005101.4(179584302..179704629, complement)	GluA2, GluR-K2, GluR2, gluR-B

glutamate receptor, ionotropic, AMPA2 (alpha 2) [*Mus musculus*(house mouse)]	Chromosome 3, NC_000069.6 (80684935..80803204, complement)	GluA2, GluR-B, Glur-2, Glur2

Largest block on chr2(37Mb) and chr3(32Mb)

DNA analysis :

Report by email from King Faisal Specialist Hospital and Research Center, Riyadh

We did the homozygosity mapping on this patient and have been sent to exome and cytoHD. The exome data is here. The largest blocks found on DNA analysis were on chr2(37Mb) and chr3(32Mb). Both contain glutamate receptor genes. I have attached the details of the genes. The samples are at the genetic lab of Dr Namik Kaya (King Faisal Specialist Hospital and Research Center, Riyadh). His e-mail address is: namikkaya@gmail.com

Ms Albandary Binbakheet, found a likely gene harboring a mutation that may be linked to the disease. But she needs to do more experiments and needs extra samples from your son as well as your healthy siblings. She needs to perform family segregation analysis and make sure that this variant is fully segregated in the family.

Please for further information contact:

Prof , Mustafa Salih Mustafa_salih05@yahoo.com

Namik Kaya, PhD , Senior Scientist Head, Neurogenetics Unit, MBC:03, Department of Genetics
Adjunct Scientist , Pathology and Laboratory Medicine Department , King Faisal Specialist Hospital and Research Centre, P.O. Box 3354, Riyadh 11211, KSA
Email: nkaya@kfshrc.edu.sa; namikkaya@gmail.com; namikkaya@yahoo.com
Tel: +966-1-464-7272 x 39612(O_ce); 38735(Lab); 32547(Secretary)
Mob: +966-506-06-6402
Fax: +966-1-442-4585

MRI Report.

Patient's Name: Mohannad Babiker.

Date: 28-2-2008

Examination: MRI Brain.

Sequences: a) Axial cuts: FSE T1 – T2 – FLAIR – T1 + GD.
b) Sagittal cuts: SE T1 – T1 + GD.
c) Coronal cuts: FSE T2 – T1 + GD.

Report:

- There is moderate bilateral cerebral brain atrophy with moderate secondary ventricular dilatation. At the periventricular parietal regions, there are multiple nodularity and to lesser extend at the centrum semi-ovalae, hypo intense in T1, hyper intense in T2 and not enhancing with I.V. contrast.
- There is atrophy of the cerebellar vermis with a degree of bilateral atrophy.
- No evidence of dominant focal parenchymal lesion or enhancing lesion with I.V. contrast & no meningeal enhancement.
- The pituitary gland & brain stem are normal.
- Normal acoustic – facial nerves bundles & no CP angle lesion.

Concolusion:

Cerebral atrophy with premature periventricular leucomalacia plus a degree of cerebellar atrophy. The features are of micro-angiopathy state; previous hypoxic or vascular insult.

Signature:
Prof.: *Elnour Abdullah.*
Consultant Radiologist.

Khartoum Children's Clinic
عيـــادة الخرطوم للأطفال
Paediatric Consultants
إستشاريي الأطفال

Tel: 0121066660 - 0155177016 emergency- 0129211133

12/2/08

To Whom it may concern

Re; Muhanad Babikir Malik
DOB; 24/9/06
Residence Omdurman Sudan
Wt 9 Kg HC: 45 cm

Muhanad Babikir was seen in the Children's clinic with hypotonic cerebral palsy. The condition started at age 1 year with a febrile illness and convulsions in El-obaid where they were visiting Muhanad was treated as malaria// dysentery, with no response. A week later parents brought the child to Khartoum where he was treated with acyclovir and IV antibiotics (Most probably ceftriaxone) as ? meningitis/ encephalitis. His fits needed Phenytoin, Phenobarbitone and IV Diazepam for control. At presentation a month later Muhanad was noted to be very floppy, with severe head lag and lumbar lordosis, Slightly increase tone in upper limbs, floppy lower limbs but increased reflexes all over and bilateral clonus of both ankles worse on the left side. Fundoscopy was difficult but essentially normal with very sluggish papillary reflexes. He did not fix and follow for us and had raising of legs in ventral suspension. We continued his antiepileptic of Phenobarbitone 45 mg/day and Clonazepam I drop (0.1mg) /day, recommended physiotherapy and feeding advice(Good gag reflex)
An MRI scan showed diffuse atrophy and a dialated cysterna magna but no other focal changes.
Since Physiotherapy mum feels he has improved slightly.
We have been guarded in out prognosis and have informed mum that we are concerned about vision as well. She may well be coming to Saudia where they usually live and I hope you can continue to follow her up with regards to development and fits.
Thanking you

Dr Munera I Khan

E_Mail:kcc_2007@hotmail.com

Ps. MRI done last week showed-Cerebral atropy with premature periventricular leucomalacia, plus a degree of cerebellar atrophy. These features are of a

microangiopathic state, due to previous hypoxia, or a vascular insult. I personally think this all goes along with the previous illness and prolonged untreated convulsions in El-obaid.
Dr M Khan

Jejunostomy Tube (J - tube) PEJ Percautenous Endoscopic Jejunostomy

A jejunostomy tube (J-tube) is a tube that is inserted directly into the jejunum, which is a portion of the small intestine. The endoscopic approach to placement is similar to the one used for the PEG tube. The only difference is that the doctor uses a longer endoscope to enter into the small intestine. Once the endoscope is in the small intestine, the doctor will once again look for the light shining onto the surface of the abdominal wall and look for the indentation into the small intestine wall when he or she presses externally on that spot. The doctor will cleanse and numb the skin, make an incision, and pass the tube through.

When to Use a Post-Pyloric Tube

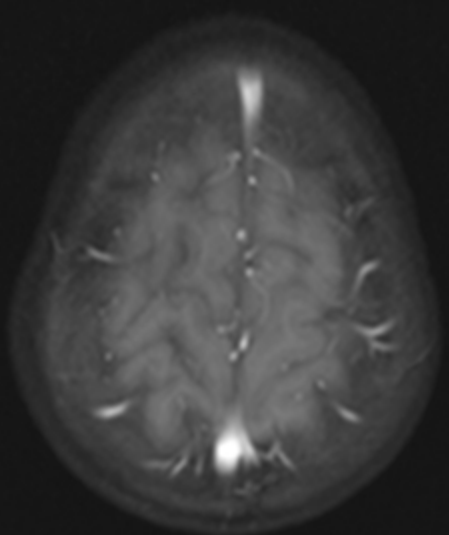
A post-pyloric tube should be considered when there is a contraindication to placing the tube directly into the stomach due to prior surgery; or when there is slow stomach emptying, due either to a mechanical blockage of the stomach outlet or to sluggish stomach motility. Examples of surgeries that make placement of a post-pyloric tube preferable or necessary include prior removal of part or all of the stomach (gastrectomy) or esophageal surgery involving removal of part of the esophagus and repositioning of the stomach into the chest to take its place (esophagectomy with gastric pull-up). In these situations, a PEG cannot be placed because the stomach is too small, absent, or sitting in the chest cavity rather than the abdomen.

Patients with slow stomach emptying due to impaired motility **(gastroparesis)** should also be considered for a post-pyloric tube. In one study comparing PEG-J and PEJ re-intervention rates over a six-month period, 56 percent of patients with the PEG-J required re-intervention compared with 13.5 percent of patients with a PEJ. Based on these numbers, it seems that a direct PEJ is the better choice.



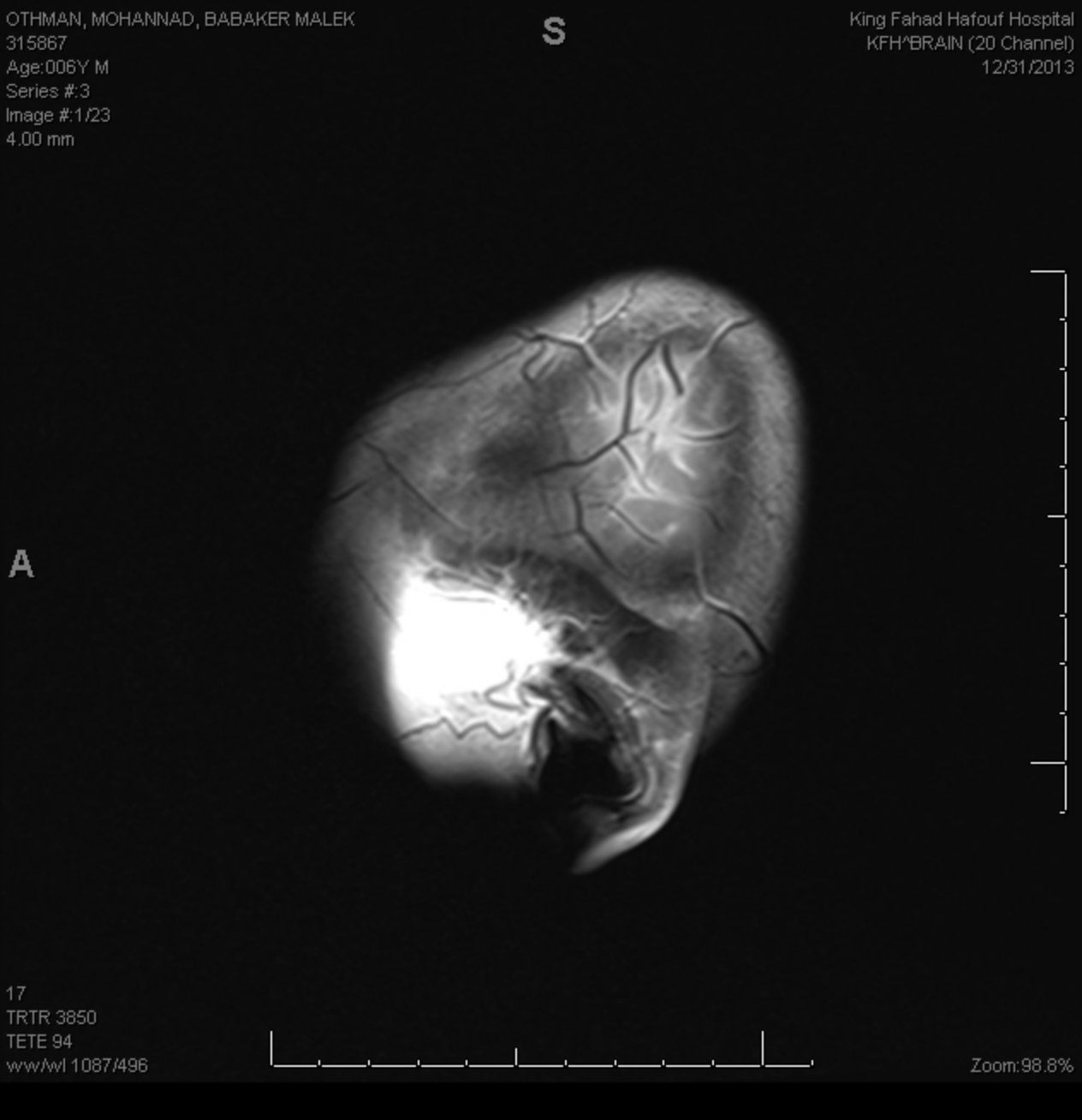
Fig. **Jejunostomy Tube (J - tube) PEJ Percautenous Endoscopic Jejunostomy size 16 Fr.**

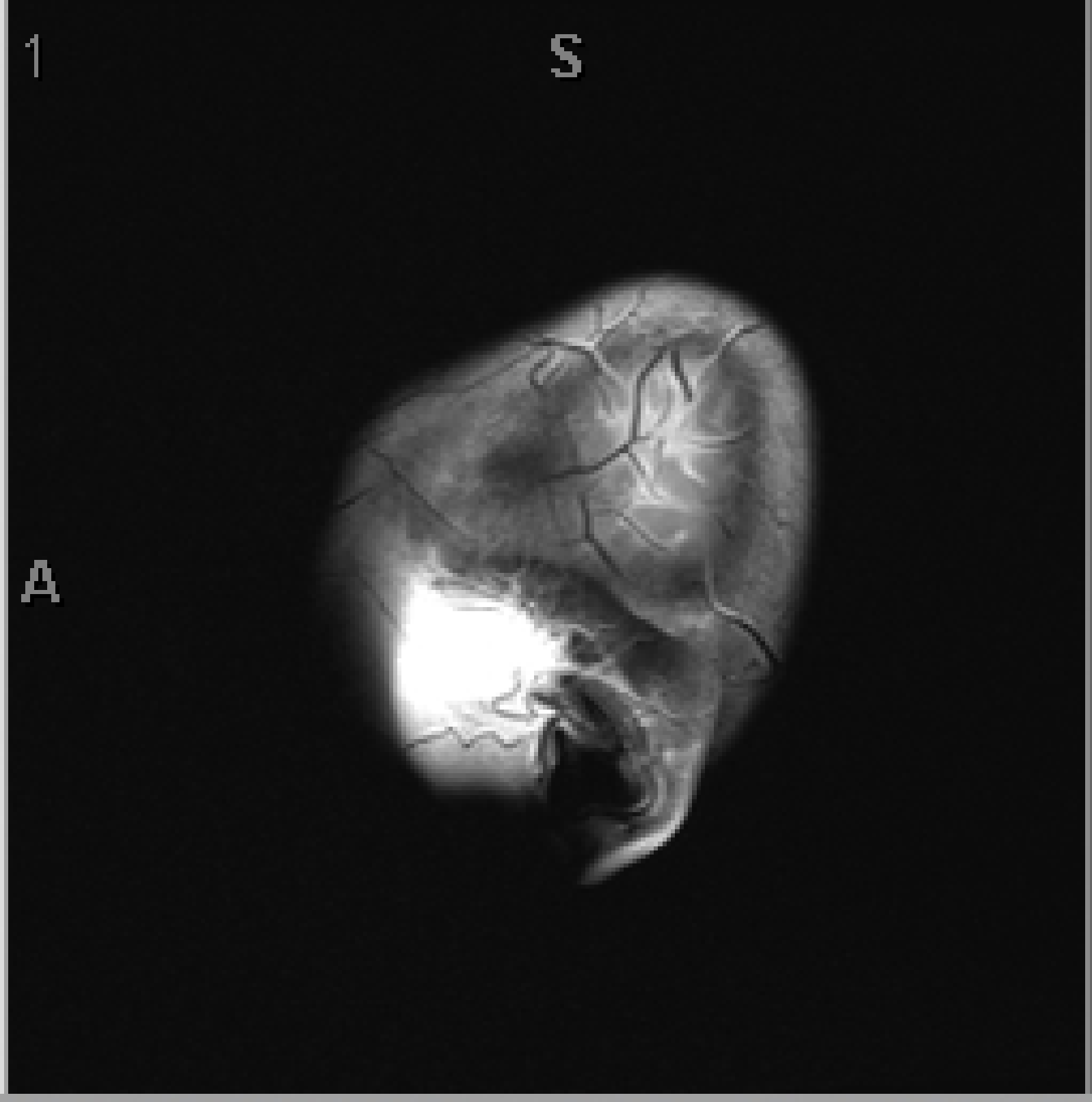
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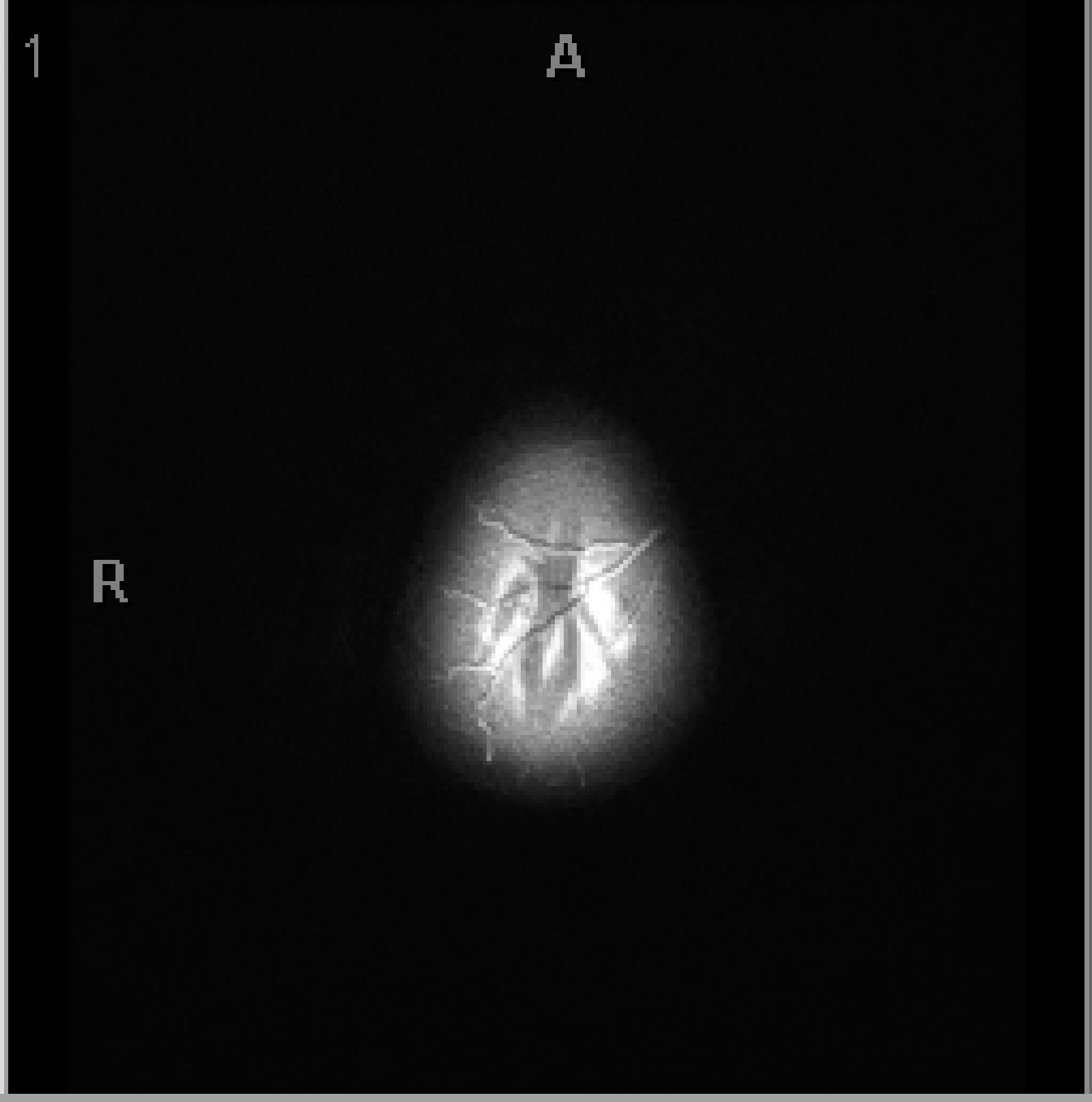
R


OTHMAN, MOHANNAD, BABAKER MALEK
315867
Age:006Y M
Series #:2
Image #:1/3
7.00 mm
S
King Fahad Hafouf Hospital
KFH^BRAIN (20 Channel)
12/31/2013
A
1
TRTR 8.6
TETE 4
ww/wl 522/234
Zoom:98.4%

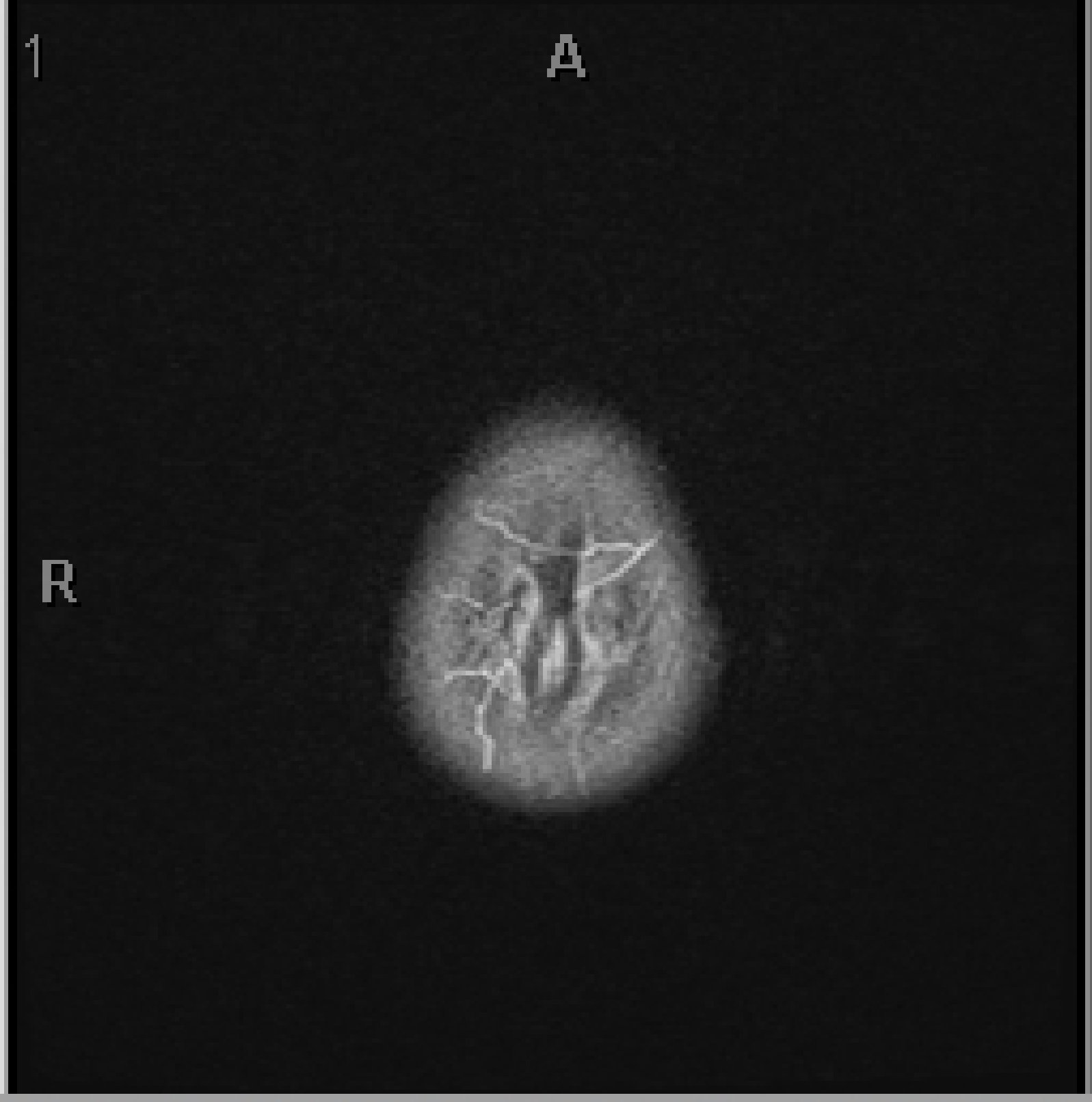

OTHMAN, MOHANNAD, BABAKER MALEK
315867
Age:006Y M
Series #:3
Image #:1/23
4.00 mm
S
King Fahad Hafouf Hospital
KFH^BRAIN (20 Channel)
12/31/2013
A
17
TRTR 3850
TETE 94
ww/wl 1087/496
Zoom:98.8%



KFH^BRAIN (20 Chan TE:94.0/TR:3850/SE
t2_tse_sag 4.0mm/5.2mm
Series #3 23 Images



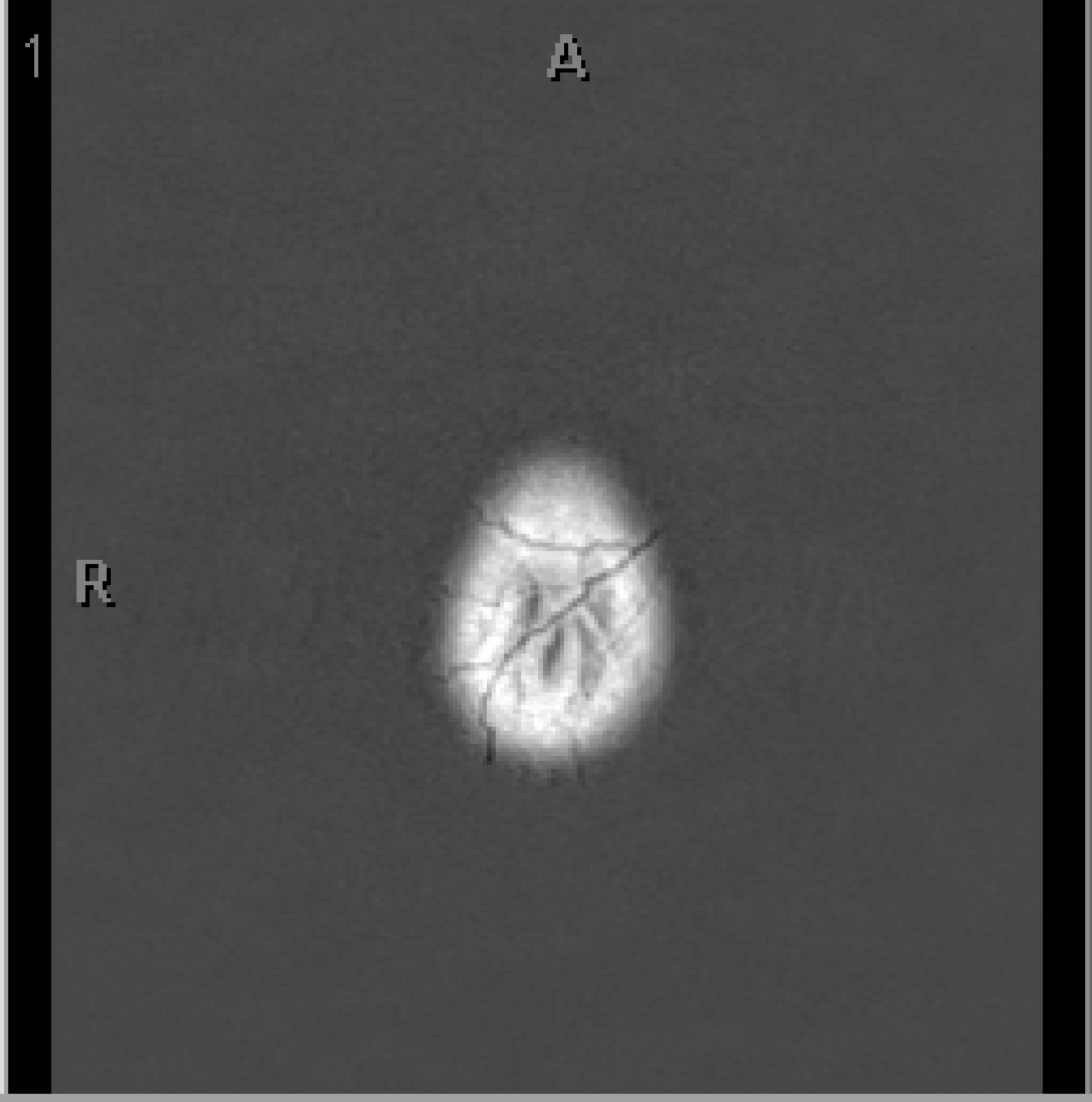
KFH^BRAIN (20 Char TE:103.0/TR:3070/SE
t2_tse_tra_512 4.0mm/5.2mm
Series #4 28 Images



KFH^BRAIN (20 Ch TE:81.0/TR:9000/SE/IR

t2_tirm_tra_dark-fluid 4.0mm/5.2mm

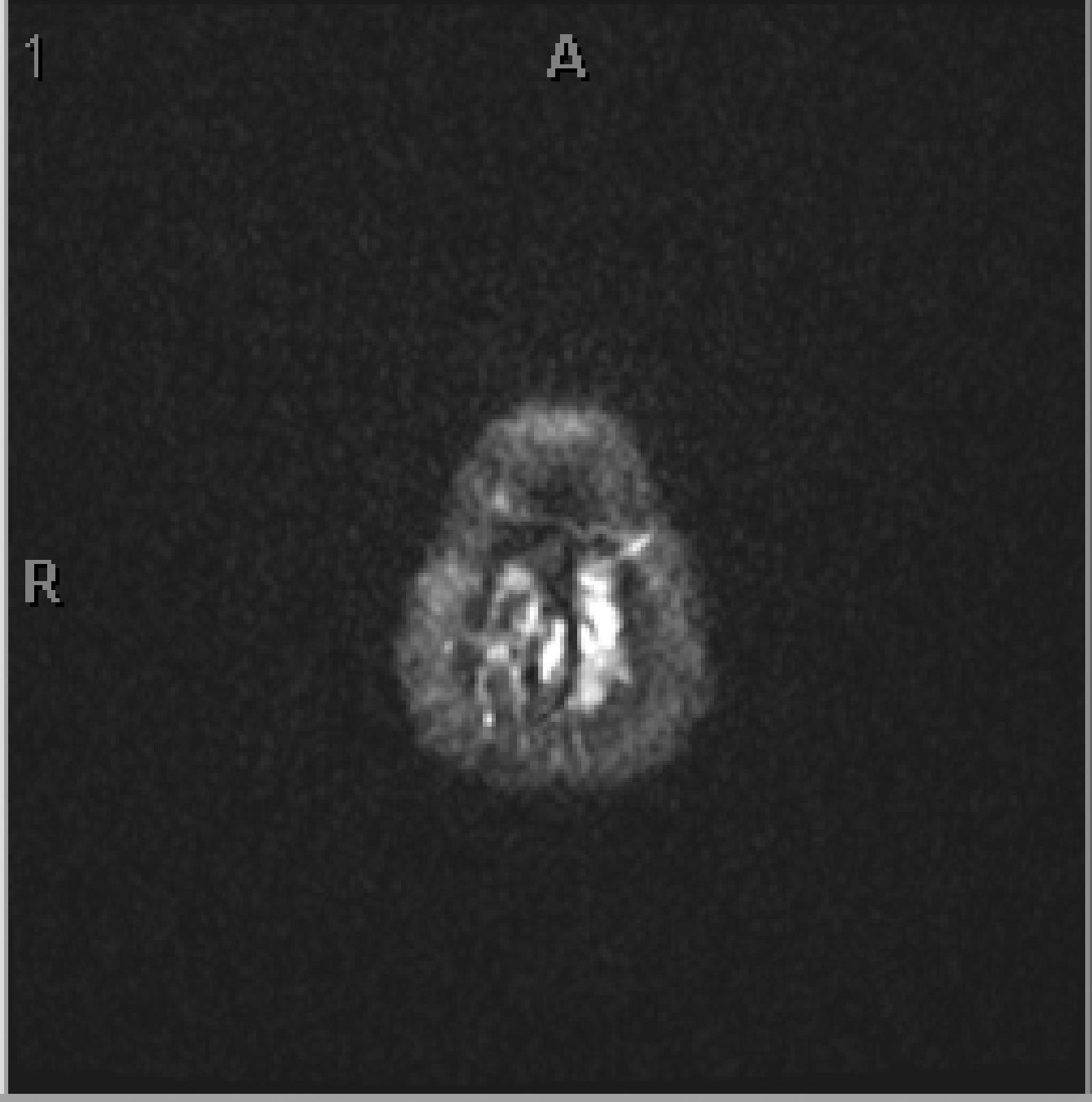
Series #5 28 Images



KFH^BRAIN (20 Ch TE:12.0/TR:2500/SE/IR

t1_tir_tra 4.0mm/5.2mm

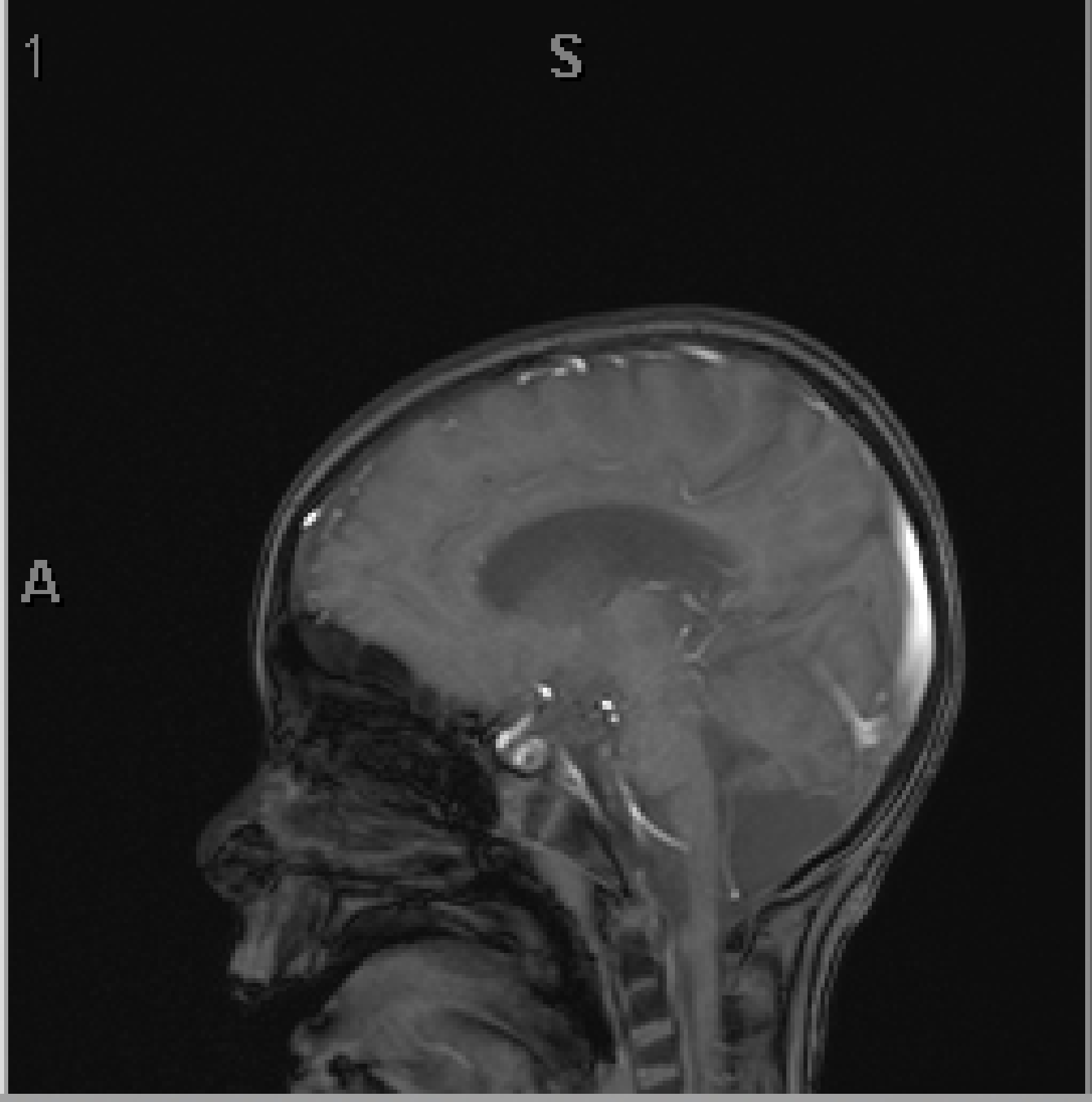
Series #6 28 Images



KFH^BRAIN (20 Chan TE:98.0/TR:6400/EP

ep2d_diff_3scan_trace_p4.0mm/5.2mm

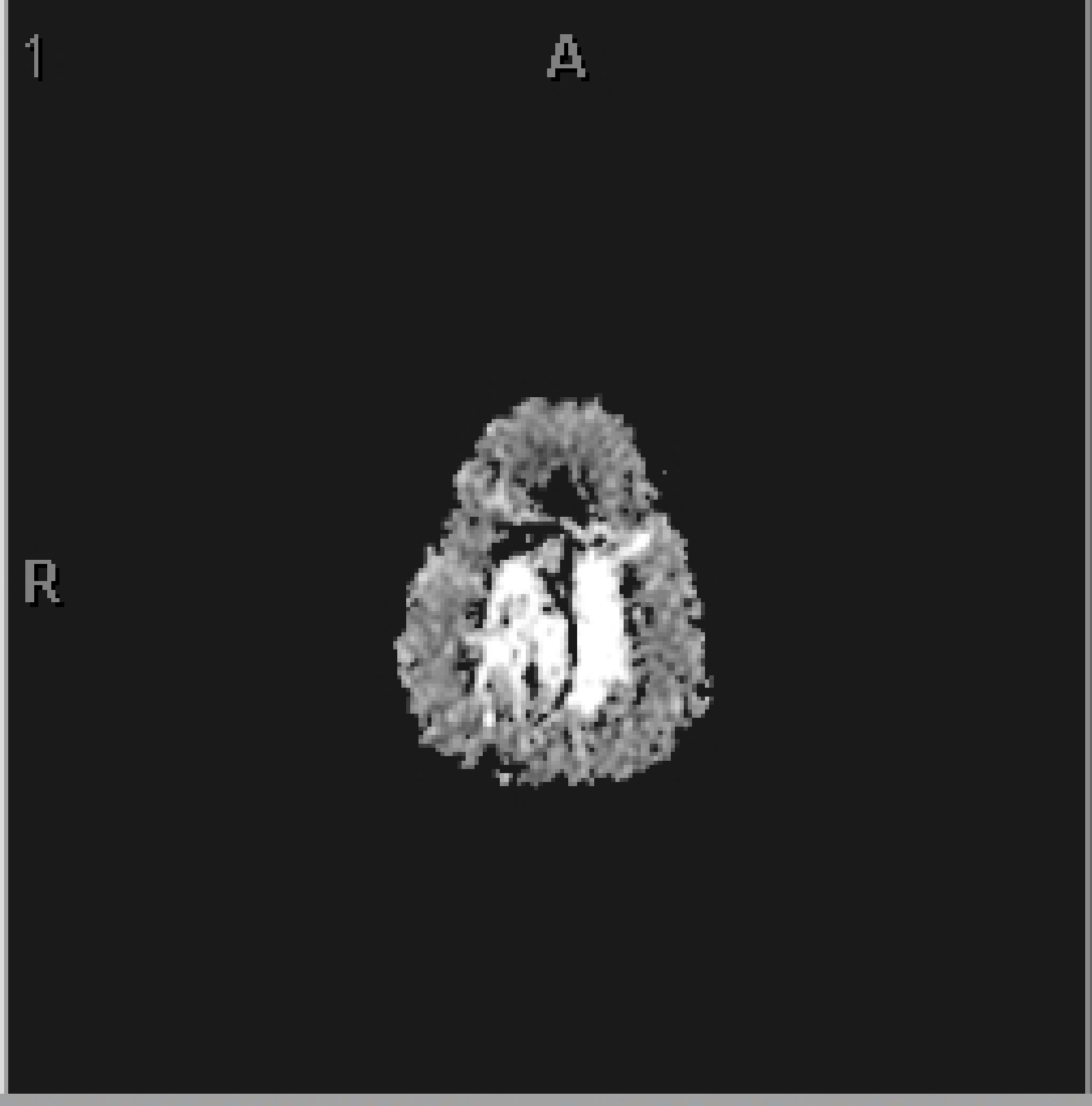
Series #7 84 Images



KFH^BRAIN (20 Channel) TE:4.0/TR:9/GR

localizer 7.0mm/8.4mm

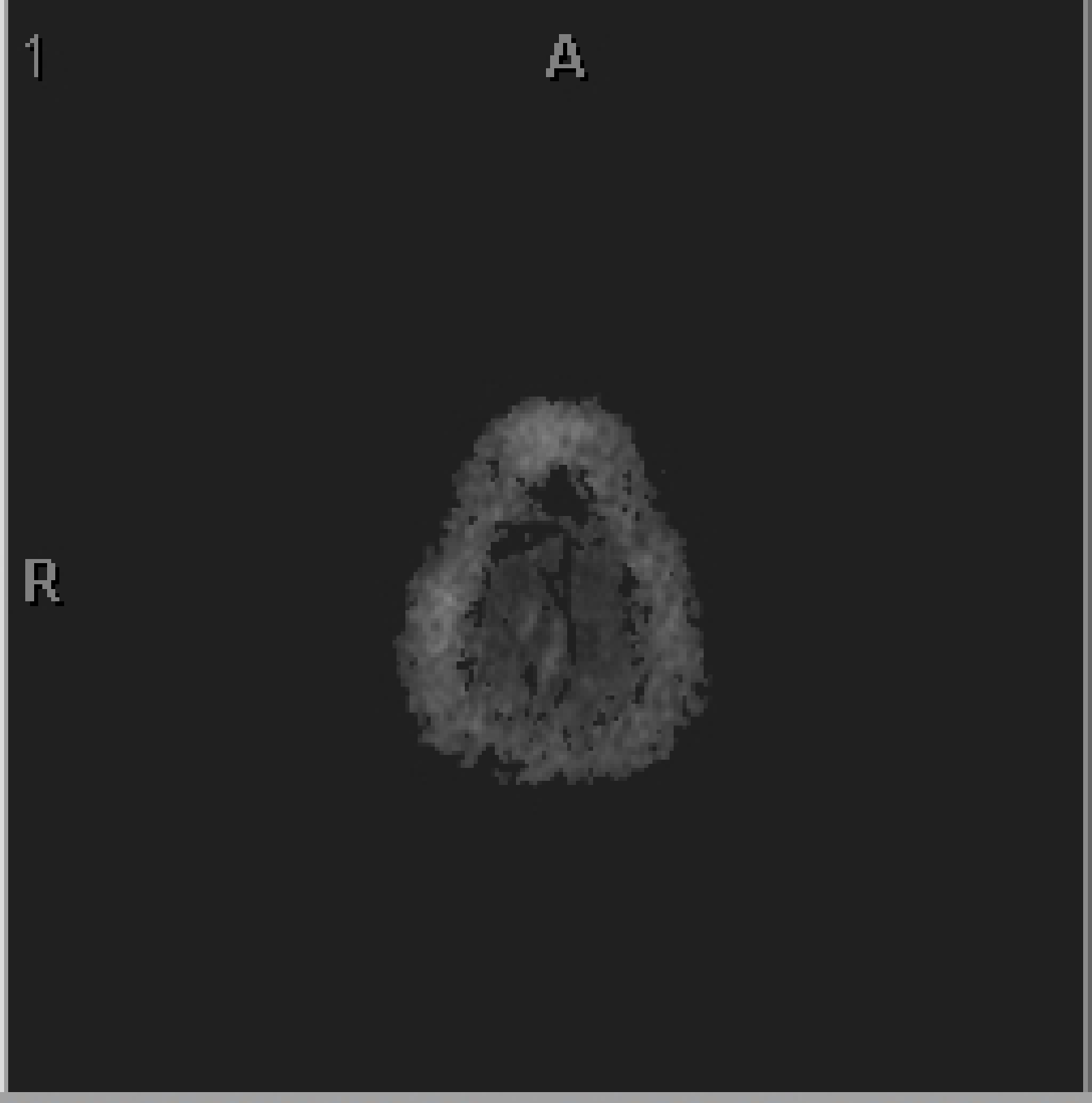
Series #2 3 Images



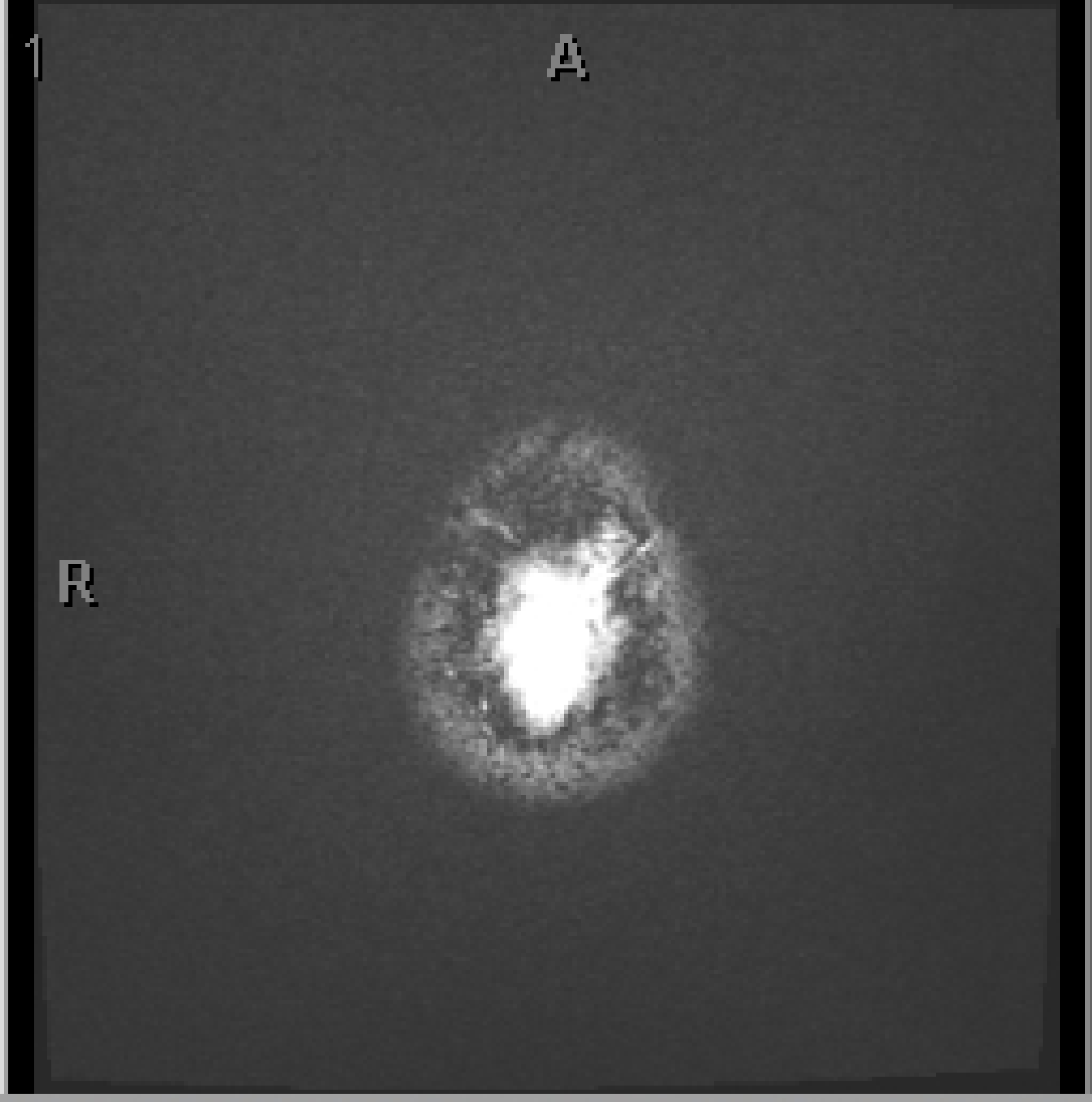
KFH^BRAIN (20 Chan TE:98.0/TR:6400/EP

ep2d_diff_3scan_trace_p4.0mm/5.2mm

Series #8 28 Images



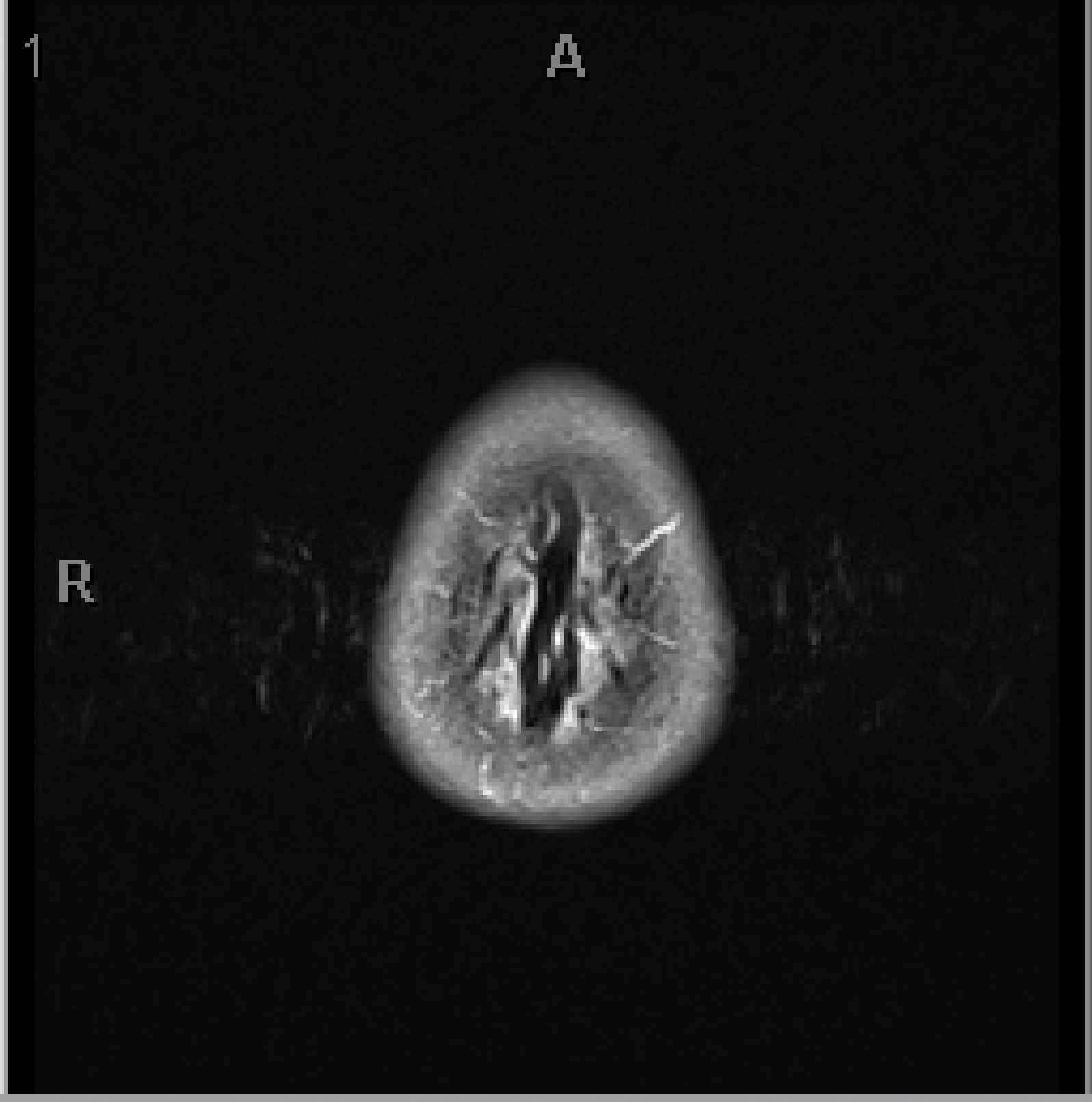
KFH^BRAIN (20 Chan TE:98.0/TR:6400/EP
ep2d_diff_3scan_trace_p4.0mm/5.2mm
Series #9 28 Images



KFH^BRAIN (20 Chann TE:19.9/TR:702/GR

t2_fl2d_tra_hemo 4.0mm/5.2mm

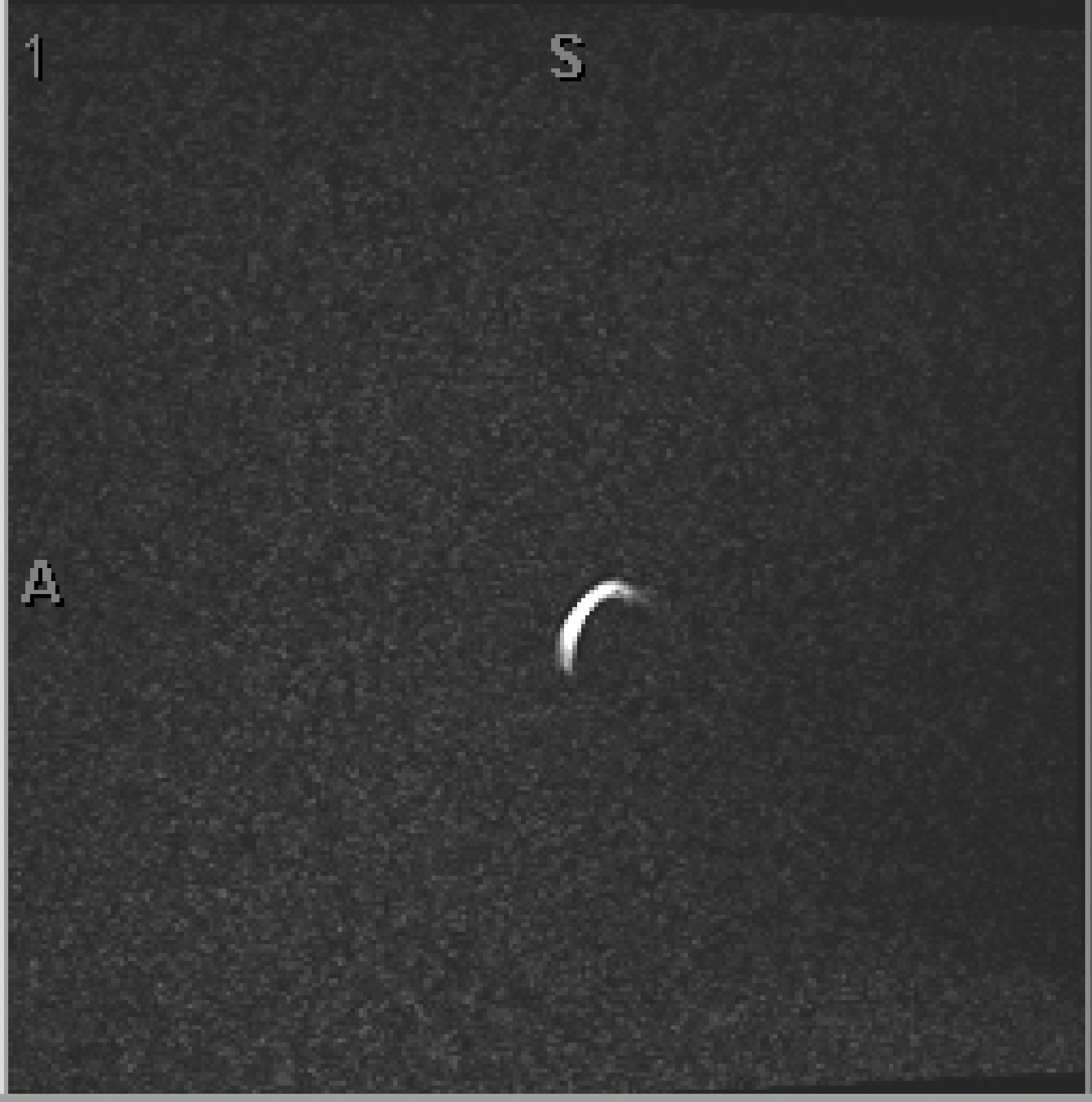
Series #10 28 Images



KFH^BRAIN (20 Channe TE:12.0/TR:812/SE

t1_se_tra-fs 4.0mm/5.2mm

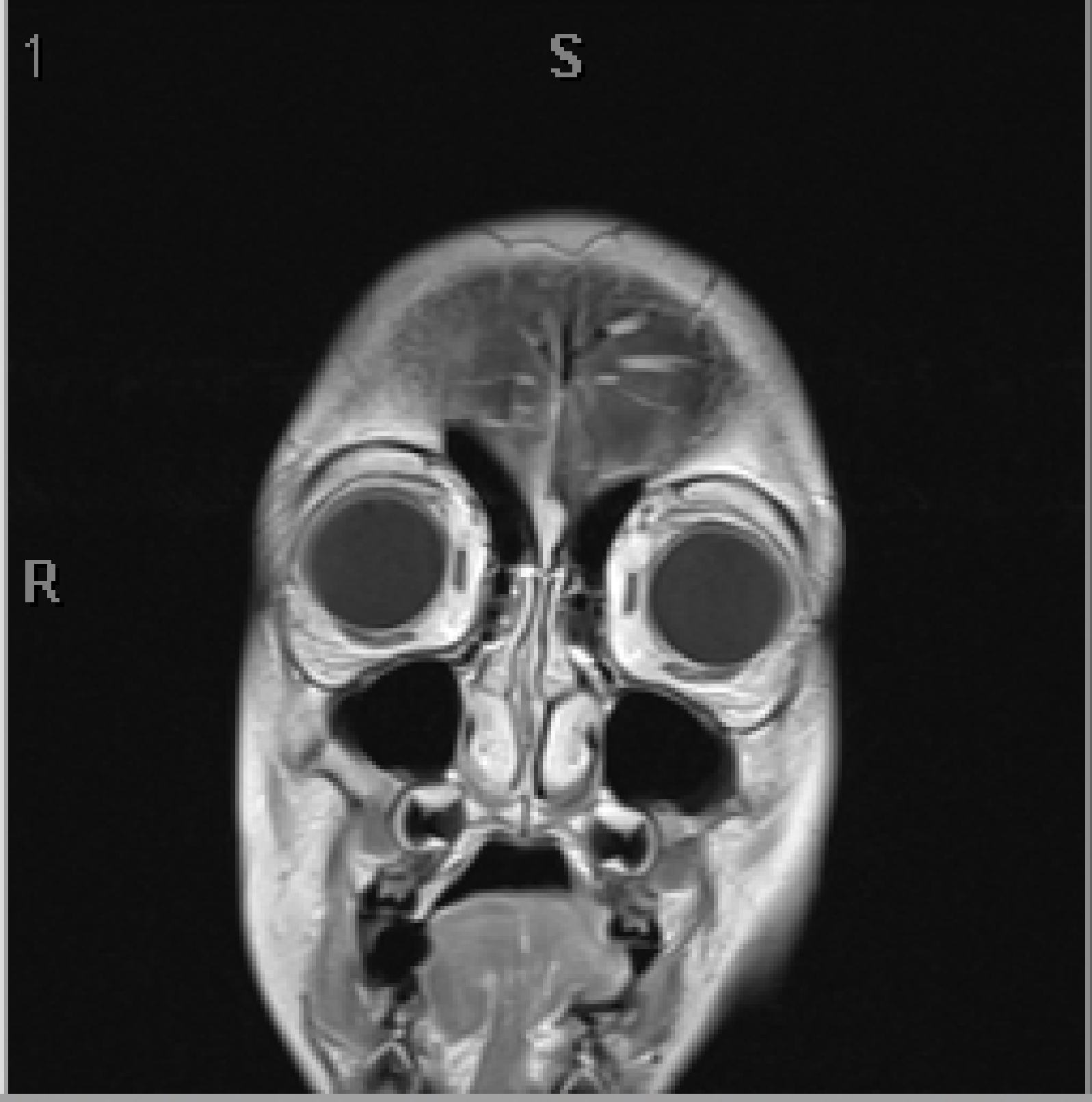
Series #11 27 Images



KFH^BRAIN (20 Channe TE:12.0/TR:812/SE

t1_se_sag-fs 4.0mm/5.2mm

Series #12 27 Images



KFH^BRAIN (20 Channel TE:6.7/TR:400/SE

t1_tse_cor_5mm 5.0mm/6.5mm

Series #13 22 Images